SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Gemcitabine and cisplatin induction before chemoradiotherapy

Head and neck: nasopharyngeal cancer · NCCP 00903 · version 1 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This chemotherapy is given before chemoradiotherapy to improve cancer control. Induction is the first stage of the planned pathway, and does not replace the later treatment.

Schedule
Gemcitabine 1,000 mg/m² on Days 1 and 8, with cisplatin 80 mg/m² on Day 1, every 21 days for a maximum of 3 cycles.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Gemcitabine
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Flu-like symptoms, fatigue, rash and swelling
- ☐ Uncommon serious lung or liver injury
- ☐ Rare haemolytic uraemic syndrome and kidney injury
- ☐ Rare severe skin reactions
- ☐ Effects on fertility; pregnancy and breastfeeding need specialist advice

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Nausea and vomiting
- ☐ Kidney injury and mineral changes
- ☐ Hearing loss or tinnitus, which can persist
- ☐ Nerve damage, which can persist
- ☐ Infusion reaction and effects on fertility

Discuss treatment aim and alternatives, expected benefit and uncertainty, all listed drug risks, infection and emergency advice, blood and organ checks, support medicines, fertility and contraception, and the option of declining treatment. Discuss kidney protection, hearing and nerve effects, and day-specific anti-sickness treatment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00903 v1.

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